St. Brazos Medical Center, Department of Radiology

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DIAGNOSTIC IMAGING REPORT

PATIENT	CHEN, David	DOB / AGE / SEX	06/11/1974 / 52 / Male
MRN	SB-7710342	ACCESSION	SBR-26-1009-4471
ORDERING PROVIDER	Alicia Moreno, MD, St. Brazos Emergency Department	EXAM DATE	10/09/2026 03:12 AM
INSURANCE	Lone Star Health, Lone Star HMO 2000	MEMBER ID	LSH-2290-1184
REPORT STATUS	Final	REPORTED	10/09/2026 04:01 AM

EXAM:

CT ABDOMEN AND PELVIS WITHOUT CONTRAST (RENAL STONE PROTOCOL)

CLINICAL INDICATION:

Acute right flank pain radiating to the groin, with hematuria. History of hypertension. Evaluate for urolithiasis.

TECHNIQUE:

Helical CT of the abdomen and pelvis was performed without intravenous or oral contrast using a low-dose renal stone protocol. Coronal and sagittal reformats were reviewed. CTDIvol: 8.4 mGy. DLP: 402 mGy-cm.

COMPARISON:

None available.

FINDINGS:

Lower chest: Lung bases are clear. Heart size is normal.

Liver and biliary: Diffusely decreased hepatic attenuation consistent with steatosis. No focal lesion identified on this noncontrast exam. Gallbladder is unremarkable. No biliary dilatation.

Spleen and pancreas: Unremarkable.

Adrenal glands: 22 mm well-circumscribed left adrenal nodule with attenuation of 28 HU (series 2, image 41). The right adrenal gland is normal.

Kidneys, ureters, and bladder: 4 mm calculus at the right ureterovesical junction with mild right hydroureteronephrosis and mild perinephric stranding. 2 mm nonobstructing calculus in the lower pole of the left kidney. No left hydronephrosis. The bladder is partially distended and otherwise unremarkable.

Bowel and appendix: Normal appendix. No bowel obstruction or bowel wall thickening.

Lymph nodes and vessels: No lymphadenopathy. Mild atherosclerotic calcification of the abdominal aorta, which is normal in caliber.

Peritoneum: No free fluid or free air.

Bones and soft tissues: No aggressive osseous lesion.

SYNTHETIC TEST DOCUMENT. Fictional patient. Hackathon prototype, not medical advice.

IMPRESSION:

1. 4 mm obstructing calculus at the right ureterovesical junction with mild right hydroureteronephrosis.
2. 2 mm nonobstructing left lower pole renal calculus.
3. 22 mm left adrenal nodule measuring 28 HU, indeterminate on this noncontrast exam. Recommend adrenal protocol CT or chemical shift MRI, and consider biochemical evaluation given history of hypertension.
4. Hepatic steatosis.

Electronically signed by: Michael Tanaka, MD
Signed: 10/09/2026 04:01 AM
Interpreting facility: St. Brazos Medical Center

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